



ICD-O-3
Carcinoma, Urothelial
8120/3

Path Site: Bladder, posterior and
Lateral Walls C67.8

Gender: Male

CPLFSite: Bladder, posterior Wall
C67.4
9w 12/24/12

Race: White

Pathology Report:

A. Right pelvic lymph node; dissection:

- Four of eleven lymph nodes, positive for metastatic carcinoma (4/11).
- Largest lymph nodal tumor focus measures 16 mm, with extranodal extension.
- Metastatic tumor includes urothelial carcinoma and small cell carcinoma components, see comment.

B. Left pelvic lymph nodes; dissection:

- Twelve lymph nodes, no tumor (0/12).

C. Urinary bladder, prostate & seminal vesicle; cystoprostatectomy:

- Invasive high grade conventional urothelial carcinoma (50%) admixed with small cell carcinoma (50%) component, invading thru the bladder wall and into the perivesical soft tissue, see pathologic parameters.
- Bladder mucosa with extensive urothelial carcinoma in situ.
- Prostate with prostatic adenocarcinoma, Gleason 3+4=7, with focal extraprostatic extension, see pathologic parameters.
- Metastatic urothelial carcinoma in one of one perivesical lymph node (1/1).

D. Left distal ureter; dissection:

- Portion of ureter, no tumor.

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive mixed conventional urothelial carcinoma (50%) and small cell carcinoma (50%), see comment.
2. Grade of tumor: High grade.
3. Depth of invasion: Extravesicular soft tissue (macroscopic).
4. Tumor distribution: Invasive carcinoma: solitary, 5 cm, right posterior and lateral walls; In situ carcinoma: multifocal, dome, anterior, posterior and left lateral walls.
5. Ureteral margins: Negative for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Negative for tumor.

8. Lymph nodes: Negative for tumor (5/24), with extranodal extension.
9. pTNM: pT3b,N2,MX.

his Checklist utilizes the 7th edition TNM staging system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

Radical Prostatectomy Pathologic Parameters

1. Gleason Score: 3+4=7.
2. Perineural Invasion: Present.
3. Tumor Location: Peripheral zone left.
4. Tumor Volume Estimate: 15%.
5. High-grade P.I.N.: Multifocal.
6. Seminal Vesicles: Negative for tumor.
7. Extraprostatic Extension: Present.
8. Peripheral Margin: Negative for tumor.
9. Distal (apical) Margin: Negative for tumor.
10. Proximal (basilar) Margin: Negative for tumor.
11. Regional Lymph Nodes (right and left): Negative for tumor (0/24).
12. pTNM: pT3a,N0,MX.

is Checklist utilizes the 7th edition TNM staging system for prostatectomy of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

Comment

The lymph node metastasis contains both urothelial and small cell carcinoma components. The urothelial carcinoma component in the lymph node focally exhibits micropapillary feature, which is not exhibited by the urothelial carcinoma in the bladder.

Immunohistochemical stains performed show that the small cell carcinoma component is diffusely and strongly positive for synaptophysin, confirming the diagnosis. Controls appropriate.

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[] MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [] MD

Electronically Signed Out by [], MD

Clinical History:

Patient is a -year-old male with bladder cancer undergoing radical cystoprostatectomy.

Specimens Received:

- A: Right pelvic lymph node
- B: Left pelvic lymph nodes
- C: Bladder, prostate & seminal vesicle
- D: Left distal ureter

Gross Description:

The specimens are received in four containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "right pelvic lymph node". Received fresh and placed in formalin is an aggregate of rubbery lobulated yellow fibroadipose tissue measuring 9 x 5.3 x 1.9 cm. The specimen is dissected for lymph node candidates. Eleven lymph node candidates are identified measuring 0.4-3.4 cm in greatest dimension. The lymph node candidates are entirely submitted as follows:

- A1: 4 lymph node candidates
- A2: 2 lymph node candidates
- A3-A4: One lymph node candidate, bisected, each cassette
- A5-A6: One lymph node candidate, bisected, one half in each cassette
- A7-A8: One lymph node candidate, bisected, one half in each cassette
- A9-A11: One lymph node candidate, sectioned

B. The second container is additionally identified as, "left pelvic lymph nodes". Received fresh and placed in formalin is an aggregate of lobulated yellow fibroadipose tissue measuring 7.5 x 4.0 x 1.3 cm. The specimen is dissected for lymph node candidates. Multiple lymph node candidates are identified measuring 0.2-2.5 cm in greatest dimension. The lymph node candidates are entirely submitted as follows:

- B1: 4 lymph node candidates
- B2: 3 lymph node candidates
- B3-B5: One lymph node candidate, bisected, each cassette
- B6-B7: One lymph node candidate, bisected, one half in each cassette
- B8-B9: One lymph node candidate, sectioned

C. The third container is additionally identified as, "bladder, prostate and seminal vesicle". Received fresh and placed in formalin is a 285.4 g, 16.5 x 14 x 3.6 cm cystoprostatectomy specimen consisting of a 7 x 6.5 x 3 cm bladder with attached mesenteric fat and a 4.4 x 4 x 3.3 cm prostate. The right seminal vesicles measure 2.9 x 1.4 x 0.5 cm and right vas deferens measures 9 x 0.6 cm. The left seminal vesicles measures 2.5 x 1.2 x 0.7 cm and left vas deferens

measures 8 x 0.6 cm. The right ureteral stump measures 2.4 x 0.9 cm and is not probe patent to the trigone. The left ureteral stump measures 2 x 0.6 cm, and is probe patent to the trigone.

The right half of the prostate and bladder is inked blue and the left half is inked black. The bladder and prostate are opened anteriorly along the urethra. The prostate is surrounded by a thin, intact membranous capsule and sectioning demonstrates rubbery, pink-tan, peri-urethral nodularity with no discrete masses.

The opened bladder reveals a 5 x 4 cm ulcerated area located in the right posterior and right lateral wall. On cut section, a firm white-tan lesion extends through the muscularis propria into the perivesical fat with a maximum depth of 1.2 cm and is 0.5 cm from the deep margin. The surrounding bladder mucosa is edematous and pink-tan with a uniform 1.0 cm wall thickness. Gross photographs are taken.

Representative sections are submitted as follows:

C1: Left ureter resection margin

C2: Right ureter resection margin

C3-C4: Distal prostatic urethral margin (apex), shaved and perpendicularly sectioned

C5-C7: Mass, deepest extent of invasion

C8: Mass and adjacent seminal vesicle (right)

C9-C10: Mass and closest approach to deep resection margin, submitted in tandem

C11-C12: Additional sections of mass

C13: Uninvolved bladder mucosa bladder dome

C14: Uninvolved bladder mucosa anterior wall

C15: Uninvolved bladder mucosa posterior wall

C16: Uninvolved bladder mucosa left lateral wall

C17-C19: Representative right lobe of prostate submitted from apex to base

C20-C22: Representative left lobe of prostate submitted from apex to base

D. The fourth container is additionally identified as, "left distal ureter".

Received fresh and placed in formalin is a 1.2 cm in length by 0.7 cm in diameter rubbery white tube-shaped structure grossly consistent with ureter.

There is 2.4 x 1.6 x 0.6 cm of attached yellow lobulated fibroadipose tissue.

There is a stitch on one aspect of the ureter that is not further designated.

The specimen is entirely submitted as follows:

D1: Ureter, stitch side

D2: Ureter, non-stitch side

D3: Remainder of ureter

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Criteria	Yes	No
Diagnosis Discrepancy <i>mixed w/ small cell</i>		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Nod	☒	
Case is (circle) <i>Prostate</i>	☒	
Reviewer Initials <i>LM</i>	QUALIFIED	DISQUALIFIED
Date Reviewed: <i>12/18/12</i>		